

CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: JB Period: September 23 to September 30, 2024 Report Date: October 1, 2024 Coverage: Bed: 135/182h (74.2%)
Chair: 145/182h (79.7%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 8 days from Sep 23 to Sep 30 ■ HR ■ Breathing

V.High HR (#1)					#4	V.High HR (#1)
Sep 23					Sep 28	Sep 30
Sep 24					Sep 29	
Sep 25					Sep 30	
Sep 26						
Sep 27						
Sep 28						
Sep 29						
Sep 30						
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage		
33	103h	Very High Heart Rate	4	100%		

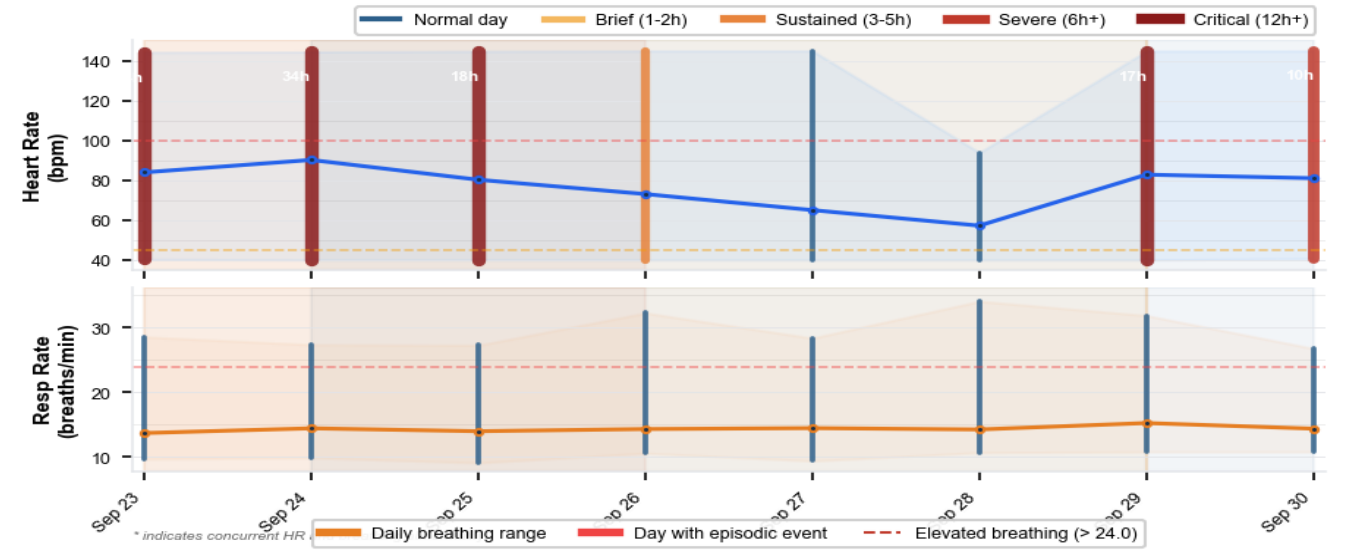
Major Findings: Sustained very high HR (avg 76, peak 145)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Very High Heart Rate	145 bpm	61h	24h	1	76 bpm	Sep 23 to Sep 29
2	High Heart Rate	145 bpm	8h	2h	2	82 bpm	Sep 23 to Sep 26
3	High Heart Rate	130 bpm	1h	1h	1	83 bpm	Sep 29
4	Low Heart Rate	40 bpm	31h	5h	2	76 bpm	Sep 24 to Sep 30
5	Elevated Heart Rate	113 bpm	1h	1h	1	83 bpm	Sep 29
6	Elevated Breathing	27 brpm	1h	1h	1	15 brpm	Sep 29

The P5 to P95 heart rate spread was 100 bpm (43 to 143), indicating significant cardiac variability.

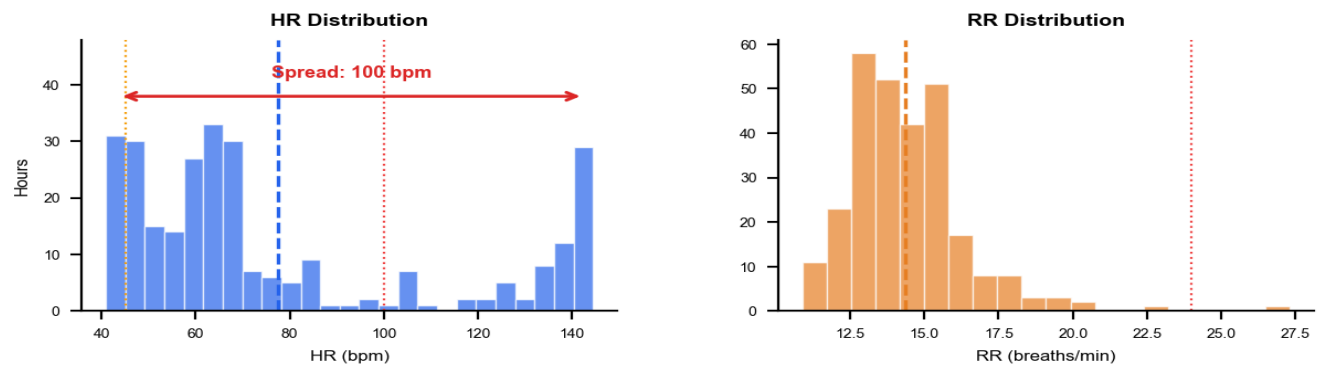
Clinical Pattern Observations:

- 1. **Sustained finding:** 24h continuous Very High Heart Rate beginning Sep 23.
- 2. **High variability:** HR spread 100 bpm (43 to 143).
- 3. **Continuous pattern:** Episodic events on 6 of 8 days (75%).

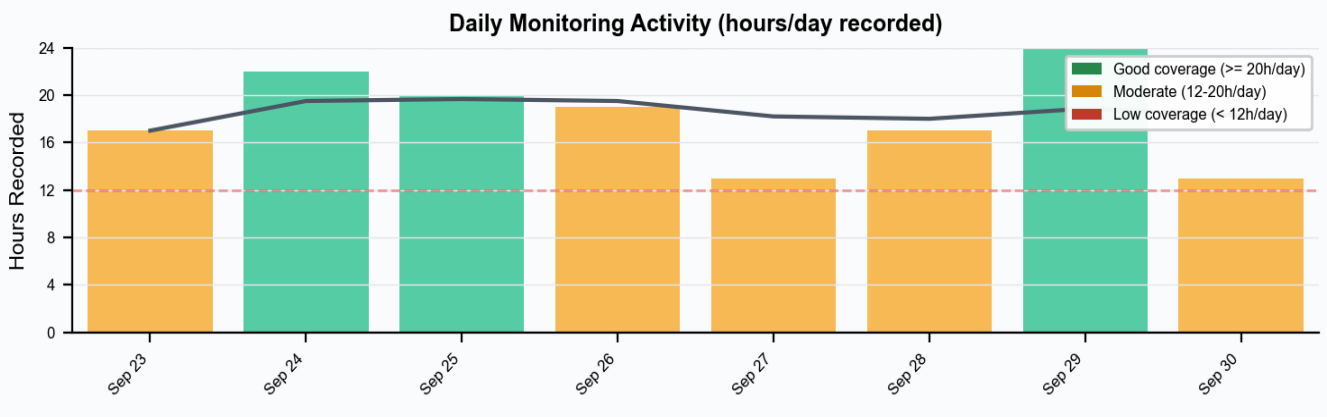


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

■ Very Low HR < 40 bpm ■ Low HR < 45 bpm ■ Elevated HR > 95 bpm ■ High HR > 100 bpm
■ Very High HR > 110 bpm ■ Elevated Breathing > 24 brpm ■ High Breathing > 30 brpm ■ Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.